

Concussion Clinical Cheat Sheet

RED FLAGS → ED TRANSFER IMMEDIATELY

- GCS ≤ 13 at any point
- Progressive ↓ consciousness
- Focal neuro deficit
- Seizure (post-injury)
- Repeated vomiting (>1)
- Severe / worsening headache
- Skull fracture signs (CSF leak, raccoon eyes, Battle's)
- Suspected cervical injury
- Anticoagulant + head injury
- Age > 65 + anticoagulant

MODIFYING FACTORS (≥3 = HIGH RISK)

- Adolescent (13–18) · female
- Prior concussion < 12 months
- Migraine / anxiety / ADHD / sleep d/o
- High initial symptom burden
- LOC > 1 min · PTA > 24 hrs
- Worsening day 2–3 · no improvement by day 7
- Legal / compensation · stress · poor support

CONCUSSION PHENOTYPES — IDENTIFY DOMINANT 2–3, DIRECT REHAB ACCORDINGLY

Vestibular — dizziness, vertigo, motion sensitivity

Affective — mood, anxiety, dysregulation

Oculomotor — tracking, screen intolerance, NPC fail

Cervicogenic — neck pain, cervicogenic HA, upper cervical

Cognitive — brain fog, slowed processing

Sleep — insomnia, non-restorative sleep, fatigue

Migrainous — pulsatile HA, photo/phonophobia

Autonomic — POTS, exertion intolerance, orthostatic

CORE CLINICAL TOOLS

SCAT6 — first 72 hrs · symptom 0–132, cognitive, mBESS

SCOAT6 — > 72 hrs · adds cervical, VOMS, autonomic

Child SCAT6 / SCOAT6 — ages 5–12

VOMS — smooth pursuit, saccades, convergence, VOR, VMS

Cervical battery — JRE, smooth pursuit neck torsion, head-neck differentiation, flex-rot, flex-ext motor control

mBESS — double, single, tandem · firm · 20 sec each · count errors

BCTT / MOVE — quantify symptom threshold · prescribe aerobic at 80–90% threshold HR

ACUTE MANAGEMENT — FIRST 72 HOURS

Day 0–2 — RELATIVE rest (not cocoon)

→ Reduce cognitive load, limit screens

→ Light ADLs as tolerated

→ Sleep priority · no alcohol/contact sport

Day 2–3+ — SUB-symptom threshold activity

→ Walking, stationary cycling

→ 15–20 min, increase as tolerated

→ Symptom flare → dial back, not stop

Analgesia — paracetamol first-line · no NSAIDs first 48 hrs · no opioids

GRADUATED RETURN LADDER · MIN 24 HRS PER STAGE · SYMPTOM FLARE = BACK 1 STAGE FOR 24 HRS

Stage	Return to Sport	Return to School	Return to Work
1	Symptom-limited activity	At home, no schoolwork	At home, no work tasks
2	Light aerobic (< 70% max HR)	Light schoolwork at home, 15–30 min blocks	Light work from home
3	Sport-specific exercise	Part-time school, quiet room, no PE	Part-time, modifications
4	Non-contact training drills	Full-time with modifications	Full-time with modifications
5	Full contact practice (medical clearance)	Full return, no modifications	Full return
6	Full return to sport	—	—

SOAP — CONCUSSION CONSULT

S — mechanism, date/time, initial + current symptoms (SCAT6 score), past concussions, modifying factors

O — SCAT6/SCOAT6 findings, VOMS, mBESS, cervical + cervico-neuro, cranial nerves

A — working diagnosis, dominant phenotype(s), red/yellow flag presence, risk stratification

P — advice, RTP/RTL/RTW stage, review timeframe, referrals, medical certificate

FOLLOW-UP CADENCE

Day 3–5 — phone review · rule out red-flag development

Week 1 — repeat symptom inventory · adjust plan

Week 2 — SCOAT6 re-administration · progress check

Week 4 — if incomplete recovery → PPCS workup + phenotype-directed plan

Beyond — fortnightly SCOAT6 until recovery · MDT as indicated